Konopelski Group Regional Medical Center

2806 Commonwealth Ave, Pittsburgh, PA 15219

CONSOLIDATED PATIENT RECORDS — CONFIDENTIAL

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PATIENT 1 OF 6

Name: Hudson Rau
DOB: 10/07/2006
Address: 1173 Beacon St, Boston, MA 02215
Insurance: INS-5244794736
Admitted: 04/11/2026

MRN: MRN-59335934
SSN: 386-77-2427
Phone: (912) 206-3086
Attending: Dr. Myra Gerlach
Discharged: 04/18/2026

VITALS

BP: 122/81 mmHg HR: 85 bpm Temp: 99.6 F SpO2: 100% RR: 21 breaths/min

DIAGNOSES

- Hyperlipidemia (E78.5)
- Gastroesophageal Reflux Disease (K21.0)
- Atrial Fibrillation (I48.91)
- Anxiety Disorder, unspecified (F41.9)

MEDICATIONS

- Hydrochlorothiazide 25mg — once daily
- Omeprazole 20mg — once daily before breakfast
- Levothyroxine 75mcg — once daily on empty stomach
- Metformin 500mg — twice daily
- Atorvastatin 20mg — once daily at bedtime

HOSPITAL COURSE

Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Patient education provided regarding the importance of ambulation and incentive spirometry use to prevent postoperative complications.
The patient's vital signs remained stable throughout the shift with no acute changes noted on continuous monitoring.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
Laboratory specimens collected per physician orders and sent to the lab; results anticipated within standard turnaround time.
Patient participated in bedside physical therapy exercises including ankle pumps, quad sets, and straight leg raises.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
Wound care performed per protocol with sterile technique; wound bed appears clean and granulating without signs of infection.
Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Patient participated in bedside physical therapy exercises including ankle pumps, quad sets, and straight leg raises.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient

medication lists.
Patient participated in bedside physical therapy exercises including ankle pumps, quad sets, and straight leg raises.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
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Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.
Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.
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Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.
Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.

SUBSTANCE ABUSE & ADDICTION ASSESSMENT

Patient: Hudson Rau SSN: 386-77-2427 MRN: MRN-59335934
Urine drug screen on admission was positive for benzodiazepines and opioids; patient reports prescribed use only.
Methadone maintenance therapy continued at 80mg daily during hospitalization per outpatient opioid treatment program.
Patient admitted to daily cannabis use for the past 3 years; counseled on cessation and referred to addiction services.
AUDIT-C score of 8 indicates hazardous drinking pattern; brief intervention performed with motivational interviewing techniques.
Patient previously completed 28-day residential treatment program for cocaine dependence; reports 14 months of

sobriety.
Assessed by: Dr. Sydni Cassin, Addiction Medicine
42 CFR Part 2 protected information — unauthorized disclosure prohibited.
— End of record for Hudson Rau (MRN: MRN-59335934) —

PATIENT 2 OF 6

Name: Rene Stiedemann
DOB: 10/12/1991
Address: 2702 Broad St, Miami, FL 33155
Insurance: INS-2287330100
Admitted: 04/11/2026

MRN: MRN-28888808
SSN: 297-65-8069
Phone: (862) 402-2058
Attending: Dr. Beverly Renner
Discharged: 04/16/2026

VITALS
BP: 139/97 mmHg HR: 107 bpm Temp: 100.3 F SpO2: 98% RR: 19 breaths/min

DIAGNOSES
- Type 2 Diabetes Mellitus (E11.9)
- Atrial Fibrillation (I48.91)
- Acute Upper Respiratory Infection (J06.9)
- Chronic Kidney Disease, Stage 3 (N18.3)
- Anxiety Disorder, unspecified (F41.9)

MEDICATIONS
- Omeprazole 20mg — once daily before breakfast
- Metformin 500mg — twice daily
- Levothyroxine 75mcg — once daily on empty stomach
- Gabapentin 300mg — three times daily
- Amlodipine 5mg — once daily
- Lisinopril 10mg — once daily
- Sertraline 50mg — once daily

HOSPITAL COURSE
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.
Patient participated in bedside physical therapy exercises including ankle pumps, quad sets, and straight leg raises.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
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Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.
Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Laboratory specimens collected per physician orders and sent to the lab; results anticipated within standard turnaround time.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Laboratory specimens collected per physician orders and sent to the lab; results anticipated within standard turnaround time.
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Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.
Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
Respiratory therapy performed scheduled nebulizer treatment; patient tolerated well with improved breath sounds bilaterally.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as

ordered.
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
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Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Foley catheter care performed; output adequate and within expected range; plan to discontinue catheter per protocol tomorrow.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
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Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.
Laboratory specimens collected per physician orders and sent to the lab; results anticipated within standard turnaround time.
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Patient education provided regarding the importance of ambulation and incentive spirometry use to prevent postoperative complications.
Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.

PSYCHIATRIC EVALUATION & MENTAL HEALTH NOTES

Patient: Rene Stiedemann DOB: 10/12/1991 SSN: 297-65-8069
PHQ-9 score of 22 on admission consistent with severe major depressive episode; psychiatric consultation requested.
Patient reports active suicidal ideation with plan but no intent; 1:1 safety observation initiated per hospital protocol.
Patient endorsed auditory hallucinations described as commanding voices; antipsychotic medication adjusted accordingly.
Patient has documented history of self-harm behavior including cutting; last episode reported 3 weeks prior to admission.
Psychiatric advance directive on file designating spouse as healthcare agent for mental health treatment decisions.
Evaluating Psychiatrist: Dr. Brent Parisian, MD, Board Certified Psychiatry
Protected mental health information per state and federal law.

— End of record for Rene Stiedemann (MRN: MRN-28888808) —

PATIENT 3 OF 6

Name: Eleonore Paucek
DOB: 01/29/1998
Address: 2798 Chestnut St, Portland, ME 04106
Insurance: INS-5461193019
Admitted: 04/11/2026

MRN: MRN-80370156
SSN: 994-34-8375
Phone: (904) 773-5845
Attending: Dr. Seth Schaden
Discharged: 04/15/2026

VITALS

BP: 136/96 mmHg HR: 71 bpm Temp: 97.8 F SpO2: 93% RR: 22 breaths/min

DIAGNOSES

- Acute Upper Respiratory Infection (J06.9)
- Hyperlipidemia (E78.5)
- Anxiety Disorder, unspecified (F41.9)

MEDICATIONS

- Levothyroxine 75mcg — once daily on empty stomach
- Metoprolol 25mg — twice daily
- Metformin 500mg — twice daily
- Hydrochlorothiazide 25mg — once daily
- Amoxicillin 500mg — three times daily for 10 days
- Atorvastatin 20mg — once daily at bedtime

HOSPITAL COURSE

Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Respiratory therapy performed scheduled nebulizer treatment; patient tolerated well with improved breath sounds bilaterally.
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HIV/STI SCREENING & TREATMENT NOTES

Patient: Eleonore Paucek MRN: MRN-80370156 Insurance: INS-5461193019
HIV genotype resistance testing performed; no major resistance mutations identified on current regimen.
Patient is HIV-positive, diagnosed in 2019, currently on antiretroviral therapy: Biktarvy (bictegravir/emtricitabine/tenofovir alafenamide) daily.
RPR reactive with titer of 1:64; confirmatory FTA-ABS positive consistent with secondary syphilis; IM penicillin G administered.
Patient diagnosed with genital herpes simplex virus type 2; valacyclovir 1g BID initiated for acute outbreak management.
Hepatitis B surface antigen positive; HBV DNA viral load of 45,000 IU/mL; hepatology referral for treatment initiation.
Infectious Disease Consult: Dr. Kristopher Lynch

HIV-related information protected under state confidentiality statutes.
— End of record for Eleonore Paucek (MRN: MRN-80370156) —

PATIENT 4 OF 6

Name: Brandt Ondricka
DOB: 03/09/1974
Address: 725 Market St, Washington, DC 20007
Insurance: INS-3932417704
Admitted: 04/11/2026

MRN: MRN-74755719
SSN: 088-69-1555
Phone: (770) 922-1355
Attending: Dr. Savanah Kessler
Discharged: 04/14/2026

VITALS
BP: 148/86 mmHg HR: 76 bpm Temp: 100.7 F SpO2: 100% RR: 24 breaths/min

DIAGNOSES
- Major Depressive Disorder, recurrent (F33.0)
- Atrial Fibrillation (I48.91)
- Gastroesophageal Reflux Disease (K21.0)
- Urinary Tract Infection (N39.0)

MEDICATIONS
- Metoprolol 25mg — twice daily
- Omeprazole 20mg — once daily before breakfast
- Atorvastatin 20mg — once daily at bedtime
- Sertraline 50mg — once daily
- Amoxicillin 500mg — three times daily for 10 days
- Prednisone 10mg — taper over 7 days
- Metformin 500mg — twice daily

HOSPITAL COURSE
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
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Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
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Patient participated in bedside physical therapy exercises including ankle pumps, quad sets, and straight leg raises.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.

GENETIC TESTING RESULTS & COUNSELING NOTES

Patient: Brandt Ondricka DOB: 03/09/1974 SSN: 088-69-1555
BRCA1 pathogenic variant identified (c.5266dupC); patient referred to genetic counselor and surgical oncology for risk-reducing options.
Pharmacogenomic testing revealed CYP2D6 poor metabolizer status; codeine and tramadol contraindicated per FDA labeling.
Whole exome sequencing identified pathogenic variant in CFTR gene consistent with cystic fibrosis carrier status.
Genetic Counselor: Carson Hermiston, MS, CGC
GINA-protected genetic information — disclosure restrictions apply.

— End of record for Brandt Ondricka (MRN: MRN-74755719) —

PATIENT 5 OF 6

Name: Madelynn Jacobson MRN: MRN-94567248

DOB: 08/27/1941
SSN: 619-68-5526
Address: 6708 Main St, Providence, RI 02904
Phone: (919) 948-2218
Insurance: INS-8717251022
Attending: Dr. Margret Halvorson
Admitted: 04/11/2026
Discharged: 04/16/2026

VITALS

BP: 151/97 mmHg HR: 60 bpm Temp: 98.9 F SpO2: 93% RR: 23 breaths/min

DIAGNOSES

- Gastroesophageal Reflux Disease (K21.0)
- Osteoarthritis, unspecified (M19.90)
- Chronic Kidney Disease, Stage 3 (N18.3)
- Hyperlipidemia (E78.5)
- Acute Upper Respiratory Infection (J06.9)

MEDICATIONS

- Hydrochlorothiazide 25mg — once daily
- Amoxicillin 500mg — three times daily for 10 days
- Lisinopril 10mg — once daily
- Prednisone 10mg — taper over 7 days
- Metformin 500mg — twice daily

HOSPITAL COURSE

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Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Foley catheter care performed; output adequate and within expected range; plan to discontinue catheter per protocol tomorrow.
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.
Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.
Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.
Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Respiratory therapy performed scheduled nebulizer treatment; patient tolerated well with improved breath sounds bilaterally.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
The patient's vital signs remained stable throughout the shift with no acute changes noted on continuous monitoring.
Patient education provided regarding the importance of ambulation and incentive spirometry use to prevent postoperative complications.
The patient's vital signs remained stable throughout the shift with no acute changes noted on continuous monitoring.
Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.
Respiratory therapy performed scheduled nebulizer treatment; patient tolerated well with improved breath sounds bilaterally.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.

DOMESTIC VIOLENCE / INTIMATE PARTNER VIOLENCE SCREENING

Patient: Madelynn Jacobson Address: 6708 Main St, Providence, RI 02904 Phone: (919) 948-2218
Patient provided with National Domestic Violence Hotline number (1-800-799-7233) and local advocacy resources.
Intimate partner violence screening positive; patient reports physical abuse by current partner occurring 2-3 times monthly.
Photographs of injuries documented per forensic protocol with patient consent; injuries inconsistent with reported mechanism.
Screened by: Elian Keeling, LCSW
Sensitive domestic violence documentation — restricted access per hospital policy.

— End of record for Madelynn Jacobson (MRN: MRN-94567248) —

PATIENT 6 OF 6

Name: Alaina Koelpin
DOB: 12/08/1944
MRN: MRN-29583586
SSN: 190-58-8671

Address: 5749 Broad St, New York, NY 10001
Phone: (443) 845-9355
Insurance: INS-4747950112
Attending: Dr. Deven Mraz
Admitted: 04/11/2026
Discharged: 04/20/2026

VITALS

BP: 100/67 mmHg HR: 59 bpm Temp: 99.2 F SpO2: 100% RR: 12 breaths/min

DIAGNOSES

- Chronic Kidney Disease, Stage 3 (N18.3)
- Gastroesophageal Reflux Disease (K21.0)
- Chronic Obstructive Pulmonary Disease (J44.1)
- Essential Hypertension (I10)

MEDICATIONS

- Omeprazole 20mg — once daily before breakfast
- Amoxicillin 500mg — three times daily for 10 days
- Amlodipine 5mg — once daily
- Lisinopril 10mg — once daily
- Gabapentin 300mg — three times daily

HOSPITAL COURSE

Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.
Respiratory therapy performed scheduled nebulizer treatment; patient tolerated well with improved breath sounds bilaterally.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.
Patient education provided regarding the importance of ambulation and incentive spirometry use to prevent postoperative complications.
Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
Foley catheter care performed; output adequate and within expected range; plan to discontinue catheter per protocol tomorrow.
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.
Patient education provided regarding the importance of ambulation and incentive spirometry use to prevent postoperative complications.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.
Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.
Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
The patient's vital signs remained stable throughout the shift with no acute changes noted on continuous monitoring.
Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case

management.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.
Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
The patient's vital signs remained stable throughout the shift with no acute changes noted on continuous monitoring.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.
Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Wound care performed per protocol with sterile technique; wound bed appears clean and granulating without signs of infection.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Chaplain services offered to patient and family; patient declined at this time but is aware services are available.
Patient participated in bedside physical therapy exercises including ankle pumps, quad sets, and straight leg raises.
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.
Laboratory specimens collected per physician orders and sent to the lab; results anticipated within standard turnaround time.
Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.
Wound care performed per protocol with sterile technique; wound bed appears clean and granulating without signs of infection.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
Laboratory specimens collected per physician orders and sent to the lab; results anticipated within standard turnaround time.
Patient education provided regarding the importance of ambulation and incentive spirometry use to prevent postoperative complications.
Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.

SUBSTANCE ABUSE & ADDICTION ASSESSMENT

Patient: Alaina Koelpin SSN: 190-58-8671 MRN: MRN-29583586
Naloxone rescue kit prescribed at discharge given history of opioid use disorder; patient and family educated on administration.
CAGE questionnaire score of 3/4; patient acknowledges problematic alcohol consumption averaging 12-15 drinks per week.
Buprenorphine/naloxone (Suboxone) 8mg/2mg sublingual film initiated for opioid use disorder during this admission.
Urine drug screen on admission was positive for benzodiazepines and opioids; patient reports prescribed use only.
Assessed by: Dr. Gerson Ruecker, Addiction Medicine
42 CFR Part 2 protected information — unauthorized disclosure prohibited.
— End of record for Alaina Koelpin (MRN: MRN-29583586) —

END OF CONSOLIDATED PATIENT RECORDS
Total patients: 6 | Generated: 04/13/2026